

DIZZINESS

Give urgent attention to the patient with dizziness and any of:

- New sudden asymmetric weakness or numbness of face, arm or leg; difficulty speaking or visual disturbance: consider **stroke** or **TIA** → 136.
- BP < 90/60
- Pulse < 50 or irregular
- Glucose < 3 (or < 4 if diabetes) → 17
- Chest pain → 37
- Difficulty breathing, especially on lying flat with leg swelling → 135
- Recent head injury
- Unable to stand without support
- New sudden severe dizziness with nausea/vomiting, abnormal eye movements or walk

Manage and refer urgently:

- If BP < 90/60, give **sodium chloride 0.9%** 500mL IV over 30 minutes, repeat until systolic BP > 90. Continue 1L 6 hourly. Stop if breathing worsens.

Approach to the patient with dizziness not needing urgent attention:

- Ask about ear symptoms. If present → 33. If hearing loss, refer same week.
- Ask about fainting/collapse attacks. If present, do ECG. If ECG abnormal, refer same day.
- Screen for alcohol/drug use: in the past year, has patient: 1) drunk ≥ 4 drinks¹/session, 2) used illegal drugs or 3) misused prescription or over-the-counter medications? If yes to any → 142.
- Review medication: antidepressants, hypertension and epilepsy treatment, furosemide and efavirenz can cause dizziness. Discuss with doctor.
- Check Hb: if < 12 (woman) or < 13 (man), **anaemia** likely → 27.
- Check BP: if ≥ 140/90 → 132. Measure BP after lying for 5 minutes and repeat after standing for 3 minutes. Does systolic BP drop by ≥ 20 or diastolic BP drop by ≥ 10?

Yes

Orthostatic hypotension likely

- This is common in the elderly.
- If diarrhoea → 46, if vomiting → 45, if fever → 24, if poor fluid intake, encourage fluids and give **oral rehydration solution**.
- Advise patient to sit first before standing up from lying down.

No

Was patient breathing very quickly or deeply immediately before or during the collapse?

Yes

Hyperventilation likely

- Usually associated with emotional stress. May also have light-headedness, chest tightness, tingling of hands/feet and visual changes.
- Encourage to breathe at a normal rate and depth.
- Assess for stress and anxiety → 86.
- If recurrent episodes, refer to psychologist.

No

Ask about associated symptoms and length of dizziness. Is there hearing loss or tinnitus (ringing/buzzing in ear/s)?

No

Sudden dizziness lasting seconds, precipitated by head movements

Positional vertigo likely

- Reassure patient that dizziness is self-limiting and usually resolves within 6 months.
- If no neck or heart problems, doctor to perform particle repositioning (Epley) manoeuvre.
- If headaches, visual symptoms or hearing loss/tinnitus develop, refer.

Yes

Sudden dizziness lasting hours/days with nausea/vomiting. May have preceding flu-like illness.

Vestibular neuronitis likely

- Mobilise as soon as possible.
- If hearing loss/tinnitus develop or no better after 2 weeks, refer.

Refer.

- If none of the above, check TSH. If abnormal, refer to doctor.
- Refer if no cause is found, dizziness persists despite above treatment or uncertain of diagnosis.

¹One drink is 1 tot of spirits, or 1 small glass (125mL) of wine or 1 can/bottle (330mL) of beer.